

Master 08 — Developer RFP / Intervention Test Build Specification

Audience: Technical suppliers, product architects, security / IG reviewers, OTOS technical advisors.

Purpose: Specify the demo build and future-platform requirements while preventing clinical-record, EPR or automated-triage drift.

Status: PENDING CLINICAL / GOVERNANCE REVIEW.

Priority: P1.

Review needed from: Dean; technical lead; IG / DPO; security reviewer; governance lead; supplier.

Source basis: Master 08 source text, Programme Plan technical build model, data model, START replacement evidence / data section.

A4 REVIEW PAGE 1 — Purpose and non-negotiables

This RFP seeks serious technical proposals for OTOS Continuity™ as a health-adjacent, non-clinical continuity infrastructure platform.

The document must keep two scopes separate:

1. **Intervention Test build** — the first bounded build for up to 50 adults. This is the immediate supplier ask.
2. **Full Pilot / future platform** — a separate later scope, priced and governed separately.

The first build must support a bounded intervention test. It must not accidentally become a clinical system, a pathway-management system or a full pilot platform.

Non-negotiables:

- no diagnosis;
- no prescribing;

- no medication management;
- no treatment decisions;
- no EPR write-back;
- no clinical-note storage;
- no therapy-note storage;
- no automated clinical triage;
- human-in-control review;
- tokenised identity;
- consent capture;
- audit-grade receipts;
- UK-hosted infrastructure;
- role-based access;
- SMS Engine designed properly from the start;
- evidence export for the formal decision set.

OTOS exists to test whether adults whose addiction, relapse or repeated crisis may be driven by untreated or inadequately treated ADHD can be identified, clinically routed, supported through authorised partner continuity activity, followed through a whole-person Continuity Thread, and evidenced safely.

The first build must create the **Continuity Management Layer** and the user's **Continuity Thread** — not a pathway. The Layer is the tokenised, role-based, consent-led infrastructure partners and the user access. The Thread is the whole-person journey over time, built from agreed non-clinical events, touchpoints, handoffs, prompts, responses, drift / silence signals, review actions and evidence-export data. Partners keep their own service routes, records, responsibilities and clinical communication. OTOS provides the infrastructure for authorised partners to view, message, log, alert and manage their agreed part of continuity without merging service records or shifting clinical responsibility.

A4 REVIEW PAGE 2 — Required response format

Suppliers should structure responses using these headings:

1. Executive technical summary.
2. Assumptions and interpretation of scope.
3. Proposed architecture.
4. Intervention Test build scope and cost.
5. Full Pilot / future platform scope and cost.
6. SMS Engine / communications architecture.
7. Security, encryption and audit model.
8. Data model.
9. Role-based access and tenancy model.
10. Delivery plan and milestones.
11. Person-days by role.
12. Commercials: non-recurring engineering, run costs and support.
13. Risks, dependencies and exclusions.
14. Similar work / team credentials.
15. Questions for OTOS before final quote.

Suppliers must explicitly state where they believe a requirement creates clinical, IG, security, safeguarding, cost or delivery risk.

A4 REVIEW PAGE 3 — Scope A and Scope B

Scope A — Intervention Test build

The Intervention Test build tests whether a consent-led Continuity Management Layer can be provided around a small, governed user cohort, with a Continuity Thread following each user's journey over time, and whether this can be evidenced safely without creating a new clinical pathway or shared clinical record.

It should support up to 50 adults, with sensible buffer capacity if agreed.

Primary purpose:

- test whether continuity receipts work;
- test whether micro mobile check-ins can keep contact warm;

- test whether SMS delivery / failure / reply / silence can be captured;
- test whether drift / silence can be surfaced earlier;
- test whether human review can be auditable;
- test whether an evidence export can be produced;
- test whether the Continuity Management Layer and user Thread can be governed without EPR integration;
- test whether partner interfaces / authorised views can access the Layer to log touchpoints and view agreed Thread elements without merging service records;
- test whether partner-approved messages, prompts and check-ins can be delivered through the Continuity Management Layer and recorded as agreed events on the user's Continuity Thread.

This is not the full platform and not the Full Pilot.

Scope B — Full Pilot / future platform specification

The Full Pilot / future platform specification is separate from the Intervention Test build. It may include:

- multiple partners;
- configurable user journeys / service-route patterns;
- multiple touchpoint types;
- partner-specific dashboards;
- communications-provider abstraction;
- consent and preference management;
- real-world evidence outputs;
- more sophisticated analytics;
- operational governance;
- future integration readiness.

Scope B must be priced separately. It is not assumed inside the Intervention Test build budget.

A4 REVIEW PAGE 4 — Demo build requirements

The Intervention Test build should support:

- participant tokenisation;
- consent capture;
- withdrawal mechanism;
- minimal personal data;
- contact method for SMS;
- communication preferences;
- partner / source service-line field;
- ADHD clinical-route status / ADHD next-step status field where agreed;
- clinical-owner field;
- medication-status category where agreed;
- addiction-service touchpoints;
- structured support / waiting-well touchpoints;
- hospital checkpoint where agreed;
- handoff receipts;
- partner-approved SMS check-in schedules;
- partner-approved outbound SMS;
- inbound SMS where possible;
- delivery receipts;
- opt-out;
- message event log;
- inbound response handling;
- drift / silence rules;
- human-review queue;
- review decision log;
- dashboard;

- evidence export;
- audit log;
- role-based access.

The participant-token link should remain with the referring service wherever possible. OTOS should not require NHS number, date of birth or full clinical identity for the first test unless governance explicitly requires it.

A4 REVIEW PAGE 5 — Evidence model: thread signal, not clinical record

The build must evidence agreed non-clinical events in the user's Continuity Thread and service journey. It must not create a clinical record, shared EPR or new pathway.

START strengthens the need to monitor the ADHD-addiction sequence together, but the system must remain non-clinical.

The build must capture:

- whether agreed thread events happened;
- whether communication landed;
- whether silence or drift emerged;
- whether human review occurred;
- whether support and addiction touchpoints were visible;
- whether a Week-12 evidence export can support a continue / continue with changes / rebuild / pause / stop decision.

The build must not store:

- clinical notes;
- therapy notes;
- prescribing instructions;
- diagnostic records;
- medication dose advice;
- full service records;
- EPR content;

- private session content;
- automated clinical-risk decisions.

The Continuity Management Layer is shared continuity infrastructure, not a shared clinical record. The Continuity Thread follows the user journey through agreed non-clinical events.

A4 REVIEW PAGE 6 — Minimum data model

The build should support these minimum entities or equivalent structures:

Entity	Purpose
ParticipantToken	Tokenised continuity subject
ConsentRecord	Consent and withdrawal
ContactMethod	SMS contact route
CommunicationPreference	Cadence, opt-out and template permissions
Cohort	Demonstrator cohort
PartnerOrganisation	Partner / service line
StaffUser	Staff access
Touchpoint	Service or support contact
HandoffReceipt	Next-step receipt
CheckInSchedule	Partner-approved planned prompts
CheckInResponse	Participant reply
DriftSignal	Silence / stuck signal
HumanReview	Review workflow
ReviewDecision	Decision log
MessageTemplate	Partner-approved SMS text
MessageEvent	SMS audit
SmsDeliveryReceipt	Provider receipt
EvidenceExport	Reporting pack
AuditEvent	Append-only audit

The supplier may propose alternative entity names, but must preserve the boundary: continuity infrastructure and Thread events only, no clinical record.

A4 REVIEW PAGE 7 — Dashboard and evidence export

The dashboard is a role-based view of agreed Layer activity and authorised Thread events. It is not a clinical dashboard and must not imply full visibility across partner records.

The first dashboard should include:

- participants onboarded;
- active participants;
- consent complete / withdrawn;
- handoff receipts created;
- handoffs confirmed as landed;
- open drift signals;
- human reviews completed;
- median time to review;
- SMS sent / delivered / failed / replied / opted out;
- check-in completion rate;
- unresolved items;
- partner burden indicator.

Required evidence export categories:

1. cohort summary;
2. consent / withdrawal summary;
3. ADHD clinical-route / next-step status summary where agreed;
4. medication-status category summary where agreed;
5. addiction-service touchpoint report;
6. structured support / waiting-well touchpoint report;
7. handoff receipt report;
8. SMS delivery / failure / reply / silence report;
9. drift / silence report;

10. human-review report;
 11. partner burden report;
 12. safety / incident report;
 13. evidence-labelled claims table;
 14. continue / continue with changes / rebuild / pause / stop recommendation.
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A4 REVIEW PAGE 8 — Security, IG and exclusions

The supplier must describe:

- hosting location;
- encryption at rest;
- encryption in transit;
- secrets management;
- role-based access;
- audit trail;
- user provisioning and de-provisioning;
- backup and restore;
- incident response;
- data retention and deletion support;
- SMS provider security;
- sub-processor position;
- support model;
- monitoring;
- logging;
- vulnerability management.

The first build should not require:

- EPR integration;
- NHS number matching;
- clinical-note storage;

- prescribing workflow;
- diagnostic workflow;
- AI clinical triage;
- full multi-tenant platform;
- national rollout architecture.

The first build should provide the Continuity Management Layer for the intervention test only; it must not become a full pathway-management platform by accident.

It should be built in a way that does not block future scale if the evidence supports continuing.

Review notes

Dean decision needed

☒ ~~Confirm whether the RFP asks for demo build only or demo plus future platform pricing.~~

Dean note: Ask suppliers to separate demo build pricing from future platform pricing. Scope A and Scope B must stay separate.

☒ ~~Confirm whether the technical brief separates the Intervention Test from the Full Pilot.~~

Dean note: Yes. The immediate supplier ask must be the Intervention Test build. The Full Pilot / future platform is a separate later scope, priced and governed separately. Do not let the first build become a full pilot, pathway-management system or clinical platform.

☒ ~~Confirm whether buffer capacity beyond up to 50 adults remains in the technical brief.~~

Dean note: Keep sensible buffer capacity if it does not distort the first-test scope, but the first demonstrator remains up to 50 adults.

☒ ~~Confirm whether SMS provider preference is known or left open.~~

Dean note: Leave SMS provider open for supplier recommendation unless governance, cost, UK hosting, deliverability or procurement requirements dictate a preferred provider.

- ☒ ~~Confirm whether supplier should price evaluation support or export only.~~

Dean note: Supplier should price evidence export clearly. Evaluation support can be optional / separately priced so it does not blur technical delivery with evaluation ownership.

Evidence / citation check needed

- ☒ ~~None required for technical procurement draft unless evidence claims are retained.~~

Dean note: Tick. No evidence citation is needed for the technical procurement draft unless evidence claims are retained in supplier-facing copy.

Clinical / governance check needed

- ☐ IG / DPO review required before supplier circulation.

Dean note: Leave unticked for now. This needs IG / DPO input before approval.

- ☐ Security review required.

Dean note: Leave unticked for now. This needs security / technical-governance input before approval.

- ☐ Clinical boundary review required.

Dean note: Leave unticked for now. This needs clinical / governance input before approval.

- ☐ Confirm medication-status category fields.

Dean note: Leave unticked for now. This needs clinical-team input before approval.

- ☐ Confirm human-review workflow and safeguarding escalation fields.

Dean note: Leave unticked for now. This needs safeguarding / governance input before approval.

Copy approval

- ☐ Approved
- ☐ Needs edit
- ☐ Hold

Appendix note — visual architecture reference

The technical drawing brief has been moved out of this supplier-facing RFP and should be held as a separate visual / diagram-production reference.

Use the Developer RFP above as the build brief. Use the separate visual architecture reference only to create or check diagrams that explain the build.

Appendix — Technical Drawing Brief / Visual Architecture Reference